

Gastrointestinal Problems in Eating Disorders

Gastrointestinal (GI) problems are one of the most common consequences of an eating disorder. Individuals with Anorexia Nervosa, Bulimia Nervosa, and other eating disorders may experience a number of GI changes, which are believed to be caused by starvation, malnutrition, and the underuse of the GI tract. GI problems experienced by people with an eating disorder may include:

- Bloating
- Constipation
- Diarrhoea
- Flatulence
- Abdominal pain
- Fullness after eating even very small amounts
- Gastric reflux, nausea, and vomiting



Why do people with eating disorders experience gastrointestinal problems?

There are a number of ways that eating disorders disrupt the digestive system. One way is through chronic restriction which can cause the muscles of the small and large intestine to atrophy, or waste away, due to underuse. This slows the movement of food through the digestive system (known as delayed gastric emptying) and can cause stomach aches, bloating and wind.

The fat that normally surrounds the intestines (mesenteric fat) is also reduced, which further slows digestion. When the body is in starvation mode, it produces fewer enzymes and hormones needed for digestion, which slows the process even more. Vomiting can also interfere with GI functioning as the rhythm and process of digestion is interrupted by the act of vomiting.

Normal digestion through a healthy intestinal tract can vary a lot and can take anywhere from 8-12 hours up to 23-37 hours. For someone with an eating disorder, it is not possible to predict how long digestion will take because the system is disrupted.

Electrolyte disturbances (especially changes in calcium, sodium, potassium, and magnesium) can affect how the gut muscles work. When these minerals are out of balance, the muscles of the intestines may not contract properly, which further slows down the movement of food. Inadequate and irregular food intake also affects bowel functioning. The bowel requires an adequate

amount of waste to be in it in order to empty. With limited intake, the bowel does not function effectively or regularly and it may be many days between bowel movements. A diet low in fibre and water can also contribute to poor bowel function.

A consequence of slow, inefficient digestion and poor bowel functioning is that people recovering from eating disorders often experience bloating, constipation, and pain. These symptoms are sometimes mistaken for feeling full which can make the process of renourishment even more physically and psychologically challenging.

The misuse of laxatives can also cause GI problems. Laxatives disrupt normal bowel function and can cause symptoms such as: loss of intestinal muscle tone, bloating, gas, colicky pain, appearance of mucus and blood in the stool, incontinence of faeces, and in severe cases, paralysis of the bowel. See our handout titled [Laxative Misuse](#). In most people, these symptoms are reversible after stopping laxatives, but some permanent effects may occur. **It is important to not self-medicate with laxatives for the relief of constipation.** Continuing to take laxatives to relieve various GI symptoms maintains these symptoms as our digestive system does not get the opportunity to build up strength to digest food on its own.

Treating Gastrointestinal Problems

GI problems will improve when food intake and behaviours interfering with digestion are normalised. If you are working towards increasing the frequency, quantity, and/or variety of food you are eating, it is normal to expect some GI discomfort in the short-term. This does not mean you have eaten too much and is a normal part of the recovery journey for many people. Some people find it helpful to alleviate this discomfort by using a hot water bottle or engaging in a distracting activity after eating. It may take some time for the system to recover normal functioning. If you are concerned about GI problems it is important you seek professional medical advice.

